

SYNTHETIC PATIENT RECORD - SOFTWARE TESTING ONLY

Patient Medical Chart

Atlantic Vein and Vascular Center | Vascular Surgery | Florida

1. Patient and coverage information

Patient	Elena Sofia Garcia	DOB / age	1967-04-18 / 59
Member ID	FLB-SYN-1001	Plan	Florida Blue PPO (synthetic)
Chart ID	AVV-2026-1042	Encounter	2026-07-08
Clinician	Priya Nair, MD	Location	Tampa, FL

2. Prior authorization request

Procedure	Endovenous radiofrequency ablation, right great saphenous vein	CPT	36475
Diagnosis	Symptomatic venous insufficiency with right-leg varicosities	ICD-10	I87.2; I83.891
Laterality	Right	Requested date	2026-07-22
Authorization	Initial request	Payer	Florida Blue

3. Clinical history and symptoms

The patient has persistent right medial-calf aching, swelling, itching, and burning that began in January 2026 and is worse after standing. Symptoms are anatomically associated with the refluxing right great saphenous distribution.

Symptoms significantly interfere with activities of daily living. She cannot stand longer than 20 minutes to cook, has stopped her usual grocery-shopping trips, and must elevate the leg during her workday. Pain is rated 7/10 by late afternoon despite conservative care.

Examination documents rope-like right medial-calf varicosities measuring 4.2 to 5.0 mm, pitting ankle edema, and intact skin without ulceration. The treating clinician assigns CEAP class C3.

4. Venous duplex ultrasound - 2026-06-26

Finding	Result	Interpretation
Right great saphenous vein	Reflux 2.8 seconds at SFJ and proximal thigh	Pathologic saphenous reflux demonstrated
Visible tributary varicosities	4.2 to 5.0 mm	At least 3 mm
Deep venous system	Compressible; no thrombus	No DVT
Left leg	No clinically significant reflux	Not included in request

5. Conservative management

From 2026-02-01 through 2026-06-30, the patient wore prescribed 20-30 mmHg graduated compression stockings during waking hours for five months. Adherence is documented in the nursing log and at follow-up visits.

She also completed leg elevation, daily walking, weight management counseling, and as-needed acetaminophen. Persistent pain, edema, itching, and burning did not materially improve after more than three months of compression therapy.

6. Assessment and plan

Assessment: Symptomatic right great saphenous venous insufficiency with documented reflux, CEAP C3 disease, varicosities greater than 3 mm, substantial ADL interference, and failed conservative management.

Plan: Proceed with endovenous radiofrequency ablation of the right great saphenous vein. No treatment is requested for the left leg, accessory veins, perforator veins, or telangiectasia.

Attestation: This is a wholly fictional record created for software regression testing. It must not be used for clinical care, billing, or coverage determination.